

Heart Rate and Heartbeat Questions - Female (Adult)

Algorithm 1378 | Adult (18 years and older) | Gender at birth: Female

Definition

- UAT-only assessment of a fast, pounding, fluttering, slow, or irregular heartbeat across adult and pediatric patients; not approved for real-patient or production use.
- Adult patient.
- Female.

Initial assessment

1. [OPEN_TEXT] What is the patient's age, sex, pregnancy or postpartum status where relevant, measured heart rate if available, duration, pattern, and whether symptoms are occurring now?
2. [YES_NO] Is there chest pain, severe breathing difficulty, fainting, seizure, blue/grey color, confusion, severe weakness, or inability to wake normally?
3. [OPEN_TEXT] Is there congenital or other heart disease, prior arrhythmia, family history of sudden cardiac death, fever, dehydration, bleeding, thyroid disease, anemia, stimulant/recreational-drug use, or a new/recently changed medicine?

Triage and redirect questions

1. Redirect → Fainting

Was a faint or temporary loss of consciousness the main event?

A faint or temporary loss of consciousness belongs in the Fainting pathway.

1. Disposition level 100

Are palpitations present now with chest pain, severe breathing difficulty, fainting or near-fainting, seizure, blue/grey color, severe weakness, confusion, or inability to wake normally?

NHS.UK heart palpitations guidance lists these as emergency-assessment criteria. Do not drive yourself to the emergency department.

1. Disposition level 70

Do palpitations keep recurring, last several minutes or longer, or occur in a child, during pregnancy or postpartum, with fever/dehydration/bleeding, known heart disease, family history of sudden cardiac death, stimulant exposure, or a new or changed medicine?

NHS.UK guidance recommends seeing a doctor for recurring or persistent palpitations, especially with cardiac risk factors.

1. Disposition level 15

For an otherwise well adult who is not pregnant or recently postpartum, is this a brief, occasional episode with a clear benign trigger and none of the emergency, urgent, medicine, or cardiac-risk features above?

NHS.UK guidance describes palpitations from lifestyle triggers as often not needing treatment.

Advice

Emergency palpitations precautions

- Call Qatar 999 now. Do not drive or allow the patient to drive. Keep them at rest and follow dispatcher instructions; do not give food, drink, or an extra dose of heart medicine unless directed by an emergency clinician. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.
- Call Back If: worsening chest pain or breathing; loss of consciousness, seizure, or blue/grey color.

Urgent palpitations review

- Arrange prompt in-person assessment through the Qatar pathway approved for this UAT environment; an ECG and review of medicines and contributing illness may be required. Do not advise starting, stopping, or taking an extra dose of a cardiac, thyroid, stimulant, or other medicine by telephone.
- Call Back If: episodes become more frequent, longer, or continuous; chest pain, breathing difficulty, faintness, or altered responsiveness develops.

Managing lifestyle-related palpitations

- Avoid known triggers such as stress, caffeine, alcohol, smoking, recreational drugs, strenuous exercise, and sleep deprivation.
- Call Back If: palpitations become more frequent or last longer; chest pain or fainting develops.

First aid

- Call Qatar 999 now. Do not drive or allow the patient to drive. Keep them at rest and follow dispatcher instructions; do not give food, drink, or an extra dose of heart medicine unless directed by an emergency clinician. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.

References

- NHS.UK, "Heart palpitations", <https://www.nhs.uk/conditions/heart-palpitations/> (page last reviewed 17 March 2026)

Provenance

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